ACE Transitions

Support. Strength. Success.

EMPLOYEE HANDBOOK

Workforce Standards | Peer Support Services | Medicaid Compliance

Document Control	
Version	1.0
Effective Date	_____
Board Approval Date	_____
Policy Owner	Executive Director / Program Manager
Applies To	All ACE Transitions employees; designated sections also apply to temporary staff and contractors by written agreement

IMPORTANT HANDBOOK STATUS

This handbook establishes workforce expectations and operational standards. It is not an employment contract and does not create a guarantee of employment for any specific duration. ACE Transitions may revise policies as permitted by law. When a handbook provision conflicts with governing law, Medicaid policy, payer requirements, an employee benefit plan, or a written employment agreement, the controlling authority governs.

A. Document Control and Approval

This handbook should be reviewed with ACE Transitions legal counsel, payroll/HR advisors, Medicaid compliance leadership, and applicable health plans before final adoption. It is designed as an operational baseline for North Carolina peer-support and administrative staff.

Approval	Signature / Date
Board Chair / Authorized Officer	_____
Executive Director	_____
Program Manager	_____
PSS Program Supervisor / QP	_____

PAYROLL VS. MEDICAID BILLING - DO NOT CONFUSE THESE RULES

A service note that fails Medicaid documentation standards may not be submitted as a Medicaid claim and may not count toward billable productivity, incentive credit, or service-based production metrics. However, ACE Transitions will not require nonexempt employees to work off the clock. Employees must accurately report all time actually worked, including required documentation, corrections, meetings, training, supervision, and audits. Repeated documentation failures may result in retraining, removal from billable assignments, performance correction, or discipline.

CALL RECORDING CONTROL

ACE Transitions uses recording-enabled business telephone systems for designated calls. Staff may not disable, bypass, or secretly record outside approved systems. Callers must receive the approved recording notice and any consent required by applicable law. Telehealth sessions are not automatically recorded; a telehealth recording requires separate organizational authorization and documented participant consent.

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- Use ACE-approved phones, EHR, email, telehealth, and messaging systems for ACE business. Do not place PHI in personal text messages, personal email, consumer cloud storage, or unapproved AI tools.
- All designated business calls made through ACE recording-enabled systems must follow the approved recording notice/consent workflow. Do not disable recording when recording is required, and do not make hidden recordings.
- No compliant service note = no Medicaid claim. A note must connect the service to a current person-centered plan (PCP) goal, accurately describe the intervention, record the actual start/end time, and document the participant response/progress.
- Employees must never fabricate, clone, backdate, inflate, pre-document, or alter records to create billable time. Corrections must preserve the original record and show the actual correction date.
- All required meetings, supervision, compliance reviews, audits, and job-related training are mandatory. Nonexempt employees must record the time as work time.
- Professional appearance is required for office, community, home-visit, partner-site, video, and telehealth encounters. Staff must wear or display identification when directed.
- Protect participant privacy in all settings. Do not discuss participant information where it can be overheard or viewed by unauthorized persons.
- Never deliver PSS outside certification scope, make clinical determinations reserved to licensed/QP staff, or represent administrative activities as billable peer support.

1. Purpose, Scope and Employment Relationship

ACE Transitions provides behavioral-health and peer-support services through an integrated operational structure. This handbook establishes employee conduct, administrative expectations, information-security requirements, documentation standards, and workforce controls that support safe, ethical, and payer-compliant operations.

Employment with ACE Transitions is governed by applicable law and the employee's written offer, compensation notice, benefit documents, and other signed agreements. Nothing in this handbook alters legally protected rights or creates a promise of continued employment.

Management Authority

- Employees shall follow lawful directives from authorized supervisors and comply with current written policies.
- Questions involving clinical scope, medical necessity, participant risk, or treatment decisions must be escalated to the QP/licensed clinical leadership rather than resolved by administrative staff or CPSS outside scope.
- Payer requirements and NC Medicaid policy in effect on the date of service supersede outdated internal instructions.

2. Equal Employment Opportunity and Respectful Workplace

ACE Transitions provides equal employment opportunity and prohibits unlawful discrimination, harassment, retaliation, and workplace conduct that undermines safety, dignity, or professional operations. Employment decisions are based on legitimate organizational requirements, qualifications, performance, and applicable law.

- Report discrimination, harassment, retaliation, sexual harassment, bullying, or other inappropriate conduct promptly to the Executive Director, Program Manager, or alternate designated reporting channel.
- No supervisor may discourage a good-faith complaint, direct an employee not to cooperate with an investigation, or retaliate because an employee raised a compliance or workplace concern.
- Reasonable accommodations will be considered as required by applicable disability, pregnancy, and religious-accommodation laws.

3. Employment Classifications, Scheduling and Attendance

ACE Transitions may use full-time, part-time, temporary, exempt, and nonexempt classifications. Classification is determined by the organization and applicable wage-and-hour law, not by job title alone.

Scheduling and Coverage

- Employees must maintain the schedule, availability, and service coverage assigned to their position. PSS may require evenings, weekends, community visits, or other hours that reflect participant needs.
- Absences, late arrivals, missed participant contacts, or inability to provide assigned coverage must be reported to the designated supervisor as soon as practicable and before the scheduled service whenever possible.
- Employees may not transfer a participant, swap service assignments, or delegate access to another staff member without approval.

4. Timekeeping, Payroll and Hours Worked

Employees are responsible for complete and accurate time records. Falsification, off-the-clock work, proxy clocking, or knowingly inaccurate time entries are prohibited.

- Nonexempt employees must record all hours actually worked, including required documentation, chart corrections, mandatory training, staff meetings, supervision, audit preparation, audit response, and other required duties.
- Employees may not work unapproved overtime; however, all time actually worked must still be reported. Working unauthorized overtime may result in corrective action, but time records may not be falsified to conceal work.
- Required job-related training and meetings are treated as work time for nonexempt employees unless the activity satisfies all legal requirements for exclusion from hours worked.
- Exempt employees are compensated under the applicable salary arrangement and must still comply with attendance, documentation, and workload reporting requirements.

NO OFF-THE-CLOCK CORRECTIONS

If a supervisor returns a note for correction, the employee must make the correction using the proper amendment process and record the time worked if the employee is nonexempt. ACE will address quality problems through supervision and performance management - not by requiring unpaid rework.

5. Billable Service, Documentation and Payment Integrity

ACE Transitions distinguishes employee payroll from Medicaid reimbursement. Payroll compensates employees as required by wage law and their compensation arrangement. Medicaid payment is available only for covered, medically necessary, properly authorized, accurately documented services that satisfy payer requirements.

No Note / No Claim / No Billable Productivity Credit

- A service encounter shall not be submitted to Medicaid when no compliant service note exists.
- A service note that does not identify the purpose of the contact as it relates to a current PCP goal is not eligible for Medicaid claim submission until corrected, if correction is permissible and truthful.
- Administrative contacts, scheduling, staff supervision, internal meetings, general social contact, transportation, waiting time without a covered intervention, or other noncovered activities may not be relabeled as billable PSS.
- An encounter that lacks required documentation, required authorization, eligibility, credential status, or other payer prerequisites is placed on billing hold.
- If ACE uses a productivity bonus, unit-based incentive, or other service-production payment, only validated and approved billable units qualify under that plan. Base wages and legally compensable hours remain governed separately.
- If a designated recorded-call workflow applies and the required recording is unavailable, the encounter shall be placed on compliance hold unless the employee documents an approved legal, participant-consent, or technical exception. The recording requirement does not replace the Medicaid service-note requirement.

Prohibited Documentation Practices

- Backdating service orders or notes; entering a false service date; copying a prior note without accurately reflecting the current encounter; cloning participant responses; inflating time; documenting before the service occurs; documenting a service that another person delivered; modifying a record to obtain payment; or sharing login credentials.
- Using vague phrases such as “checked in,” “provided support,” or “client doing well” without a goal-linked intervention, participant response, and progress information sufficient to demonstrate the service actually delivered.
- Recording travel time, administrative time, supervision, or noncovered collateral activity as direct H0038 service time unless expressly permitted by current payer requirements.

6. Company Cell Phones and Mobile Devices

ACE Transitions may issue organization-owned mobile phones, phone numbers, or mobile applications to employees whose duties require participant, payer, partner, or workforce communication. Company devices and accounts remain ACE property and may be managed, monitored, preserved, or remotely disabled consistent with law and policy.

- Use the assigned ACE phone number and approved applications for participant and business communications. Personal phone numbers should not be used for routine participant contact.
- Keep devices secured with a passcode or biometric lock, automatic screen lock, current operating-system updates, multifactor authentication where available, and approved security controls.
- Do not store participant photographs, screenshots, downloaded records, unapproved voice memos, or PHI in the consumer photo gallery, personal cloud account, personal notes app, or unapproved application.

- Do not use ordinary SMS/MMS for PHI unless ACE has expressly approved the workflow and determined appropriate safeguards. Use approved secure messaging whenever available.
- Report a lost, stolen, compromised, or misplaced business device immediately. Do not wait until the next workday if the device contains or can access ACE systems.
- Employees may not lend a company device to family members, friends, participants, or unauthorized coworkers.
- Incidental personal use may be allowed if limited, lawful, non-disruptive, and not inconsistent with security or record-retention requirements. ACE may restrict personal use at any time.

Use While Driving

Employees may not type, text, review records, document, conduct video visits, or hold a participant-service conversation while actively operating a vehicle. Staff must safely park before engaging in participant documentation or substantive calls. Hands-free use does not override a supervisor's safety restriction.

7. Call Recording and Monitored Communications

ACE Transitions uses recording-enabled business telephone systems for designated participant, intake, quality, training, compliance, and operational calls. Recorded calls may contain protected health information and therefore must be treated as confidential records.

- Employees shall use the approved recording-enabled platform for calls that ACE designates for recording and shall not disable, evade, or manipulate the recording function.
- Before recording begins or at the start of the call, the caller must receive the ACE-approved notice and any affirmative consent required by applicable law or organizational procedure.
- North Carolina law generally permits interception with consent of at least one party, but interstate calls may implicate the law of another jurisdiction. ACE therefore requires notice/consent procedures rather than covert recording.
- If a caller declines recording, the employee must follow the approved exception workflow. The employee may not secretly record the call. Options may include an approved unrecorded line, written communication, rescheduling, or escalation to a supervisor depending on the purpose of the call.
- Employees may not download, copy, email, text, transcribe into an unapproved system, or independently retain recordings. Access is role-based and limited to authorized quality, compliance, clinical, legal, billing, or management purposes.
- Recordings may be reviewed for training, quality assurance, complaint investigation, documentation validation, safety, legal hold, payer audit, or compliance review. Employees should have no expectation that business communications made through ACE systems are private from authorized organizational review.

STANDARD RECORDING DISCLOSURE

"This call may be recorded for quality assurance, training, documentation, and compliance purposes. Before we continue, do I have your consent to proceed with the recorded call?"

If the caller does not consent, follow the approved exception process. Do not argue with the caller or continue recording secretly.

8. Telehealth and Telephonic Service Standards

Telehealth and audio-only services may be used only when allowed by the participant's payer, current NC Medicaid policy, the applicable service code, and ACE procedures. Staff must accurately document modality and may not select telehealth merely for convenience when it is inconsistent with service requirements.

- Use only ACE-approved telehealth technology and approved accounts. Do not conduct telehealth through personal social-media video, consumer messaging, or unapproved conference applications.
- Conduct telehealth from a private, professional setting where conversations cannot reasonably be overheard or viewed. Use headphones when appropriate and position screens to prevent unauthorized viewing.
- Verify participant identity and location using the approved workflow before beginning a service encounter, and confirm an emergency contact or emergency-response plan when required by clinical policy.
- Do not conduct a telehealth visit while the employee is driving, in a public restaurant, shared waiting room, open hallway, or other location that compromises privacy.
- H0038 individual PSS may be eligible for telehealth or telephonic delivery under current policy; group H0038 HQ is not telehealth/telephonic eligible under the current 8G attachment. Staff must document modality accurately so billing can apply the correct claims requirements.
- Current NC Medicaid 8G limits telehealth and telephonic audio-only time to ten percent or less of total service time provided per beneficiary per fiscal year. Staff must cooperate with utilization monitoring.

- Telehealth sessions are not automatically recorded. Recording a telehealth encounter requires prior organizational authorization, a defined business/clinical purpose, appropriate participant consent, and secure storage.

9. Training, Credentialing and Continuing Education

Employees may perform only duties for which ACE has verified required qualifications, training, credentials, supervision, and access. Maintaining required credentials is a condition of assignment to regulated or billable duties.

Timeframe	Required PSS Training / Control	Applies To
Within 30 calendar days of hire to provide service	PSS policy components review; documentation training; opioid-antagonist administration training	All PSS program staff
Within 30 calendar days	NC Peer Support Supervisor Training	PSS Program Supervisor
Within 60 calendar days	Person-Centered Thinking; PCP instructional elements with comprehensive prevention/intervention crisis-plan training	PSS Program Supervisor
Annually	Continuing education related to the population served; ACE annual compliance refreshers	All applicable staff

ACE may require additional training, including HIPAA/privacy, 42 CFR Part 2 when applicable, fraud/waste/abuse, cybersecurity, call recording, telehealth privacy, incident reporting, mandated reporting, professional boundaries, cultural responsiveness, crisis procedures, documentation, and payer-specific requirements.

- Employees must provide requested certificates or documentation by the deadline established by ACE.
- Expired, suspended, revoked, or unverified credentials may result in immediate removal from billable duties pending verification.
- Employees may not represent that they hold a license, credential, certification, degree, or specialty they do not possess.

10. Required Meetings, Supervision and Audit Participation

Compliance meetings, supervision, staff meetings, training, and audit activity are operational requirements, not optional administrative extras. Employees are expected to attend prepared, participate professionally, and complete assigned follow-up.

Activity	Minimum ACE Standard	Purpose
QP supportive/administrative supervision	At least every other week for PSS program staff; more often based on experience, risk, or need	Scope, service quality, caseload, documentation, safety, development
Peer operations huddle	Weekly or as scheduled	Assignments, barriers, follow-up, coverage, urgent operational issues
Compliance / Medicaid meeting	Monthly	Documentation trends, denials, authorization status, policy changes, corrective action
Internal documentation audit	Monthly and risk-based	Validate notes, units, goal linkage, signatures, modality, authorization, eligibility
Board / leadership compliance review	At least quarterly or as requested	Program performance, risk, audit findings, corrective-action status

- Staff shall submit requested records, explanations, attestations, or corrections within the deadline established by the auditor or supervisor.
- Employees shall not alter a record to conceal an audit finding. Corrections must follow the authorized amendment process.
- Failure to attend required supervision or compliance meetings, repeated unpreparedness, or failure to complete corrective actions may result in removal from service assignments or discipline.

11. Medicaid Documentation and Service Note Standards

For PSS, the individual who delivered the service is responsible for documenting the service. Each contact/intervention for each date of service requires a full service note. Documentation must be truthful, individualized, timely, and consistent with the service actually delivered.

Required Note Element	ACE Documentation Expectation
Participant identification	Correct beneficiary name and Medicaid identification information
Date / service / modality	Actual date, service name, contact type, and place of service

Required Note Element	ACE Documentation Expectation
Goal linkage	Purpose of contact clearly connected to a current PCP goal
Intervention	Specific description of what the CPSS did; sufficient to support the documented duration
Time	Actual start time and end time; no estimated or rounded time used to create units
Response / progress	Participant response, effectiveness of intervention, and progress/barriers toward the goal
Authentication	Date plus signature and credentials/job title of the staff member who actually provided the service

Goal-Oriented Documentation Standard

Every billable note must answer four questions: What current goal was addressed? What peer-support intervention was delivered? How did the participant respond? What changed, progressed, or remains as a barrier? A note that does not establish this connection is not ready for billing.

EXAMPLES OF NONBILLABLE OR BILLING-HOLD DOCUMENTATION

"Called client to check in." | "We talked for an hour." | "Client is doing well." | "Helped with resources." | Notes with no PCP goal, no intervention details, no participant response/progress, unsupported time, missing signature/credentials, or documentation created for a contact that did not occur.

These examples may document that some work occurred, but they do not establish a payable Medicaid PSS claim by themselves.

12. Quality Assurance, Internal Audits and Corrective Action

ACE Transitions shall conduct record reviews and internal audits of billed and proposed units. Audit activity is intended to identify errors before claim submission, detect trends, protect participants, and reduce overpayments, denials, recoupments, and fraud risk.

- During the first 90 days of a CPSS assignment, QA may review at least 10% of billed encounters or five service notes per month, whichever is greater, and may increase review based on risk.
- High-risk triggers include repeated late notes, cloned language, unsupported duration, missing goal linkage, billing after credential lapse, telehealth-limit concerns, participant complaints, unusually high utilization, duplicate times, impossible travel/time patterns, or payer denials.
- A failed audit may result in a billing hold, corrected note when permissible, voided claim, overpayment return, retraining, increased supervision, removal from assignments, or disciplinary action.
- Employees must preserve records and cooperate with payer, DHHS, legal, accreditation, and internal audits. Destruction, concealment, or falsification of records is prohibited.

13. Confidentiality, HIPAA and 42 CFR Part 2

Employees must protect confidential participant, employee, business, payer, credentialing, and financial information. Access is limited to the minimum information needed for assigned duties. Additional protections apply to substance-use-disorder records when 42 CFR Part 2 applies.

- Do not discuss participant information in elevators, hallways, restaurants, vehicles with unauthorized occupants, social media, or other areas where the conversation may be overheard.
- Do not leave records, screens, printed schedules, or identifying information visible to unauthorized persons. Lock the workstation or device when unattended.
- Use only authorized disclosures, consents, and release-of-information procedures. Do not assume family members, friends, probation officers, employers, or referral sources are automatically authorized to receive information.
- Immediately report suspected privacy incidents, misdirected messages, wrong-recipient faxes/emails, unauthorized access, lost devices, or disclosure of participant information.

14. Electronic Systems, Email, Texting and Cybersecurity

- Use individual credentials; never share passwords or authenticate another employee.
- Use multifactor authentication where configured and report unexpected MFA prompts.
- Do not upload PHI, confidential records, or internal documents into unapproved artificial-intelligence tools, public file-sharing sites, or consumer cloud services.
- Do not install unauthorized software, disable security controls, or connect unknown removable media to ACE systems.

- Use ACE email for official communications. Verify recipients before sending PHI and use approved encryption/secure-message controls when required.
- Phishing, suspicious links, unusual login alerts, ransomware warnings, or suspected account compromise must be reported immediately.

15. Dress Code and Professional Appearance

ACE Transitions uses a professional, community-appropriate business-casual standard. Appearance should communicate credibility, safety, respect, and readiness to work in offices, community settings, homes, partner locations, meetings, and virtual encounters.

Appropriate	Not Appropriate for Routine Work
Clean business-casual clothing; ACE-branded polo/shirt when authorized; neat slacks, professional jeans when approved, modest skirts/dresses, clean closed-toe or professional footwear	Pajamas, bedroom slippers, clothing with offensive/profane/drug/alcohol/sexual content, visibly soiled clothing, excessively ripped/distressed items, clothing that exposes undergarments or creates a safety concern
Visible ACE identification badge when required; weather-appropriate layers for community work; PPE when indicated	Wearing another employee's badge, obscuring required identification, or using clothing/accessories that misrepresent a credential or professional status
Professional grooming; reasonable fragrance use mindful of participant sensitivities	Strong fragrance when a participant/environment requires fragrance sensitivity; unsafe footwear or accessories for the assigned setting

ACE will consider reasonable dress/grooming accommodations for disability, pregnancy, sincerely held religious practice, and other legally protected needs. Video/telehealth encounters require the same professional upper-body appearance and an appropriate background as in-person work.

16. Participant Relationships, Boundaries and Ethical Conduct

- Maintain recovery-oriented, participant-centered relationships and use lived experience purposefully within CPSS scope.
- Do not engage in romantic, sexual, exploitative, coercive, financial, or other dual relationships prohibited by policy or ethics requirements.
- Do not borrow from or lend money to participants; solicit gifts; sell personal goods/services; arrange personal housing; or use a participant for personal benefit.
- Do not promise benefits, housing, employment, legal outcomes, placement, funding, authorization, or clinical results that ACE cannot guarantee.
- Employees shall not provide PSS to a direct supervisor and should avoid providing PSS to coworkers within the same work unit when possible, consistent with current Medicaid program safeguards.

17. Community-Based Work, Safety and Home Visits

- Follow assignment, check-in, location, and escalation procedures for community/home visits.
- PSS may not be provided in the residence of PSS staff.
- Employees must leave an environment and contact supervision/emergency resources if conditions create an immediate safety threat.
- Do not bring unauthorized persons, children, family members, pets, or friends to participant visits.
- Maintain professional boundaries regarding participant property, food, medication, weapons, transportation, cash, and personal errands.

18. Driving, Travel and Transportation

Employees assigned to community-based work must maintain any required driver's license, insurance, and reliable transportation. Employees must comply with ACE driving and mileage procedures.

- Participant transportation is not a billable component of PSS under current NC Medicaid 8G. Any transportation activity must follow separate ACE authorization, insurance, and risk-management procedures.
- Travel between work locations during the workday must be recorded consistent with ACE timekeeping and wage rules.
- Do not document or participate in telehealth while driving. Pull over and safely park before reviewing records, messaging, or conducting participant-service calls.
- Report accidents, citations affecting driving eligibility, or loss of license/insurance when driving is an assigned function.

19. Incident Reporting, Emergencies and Mandated Reporting

- Immediately follow ACE emergency procedures for imminent danger, suicidal/homicidal risk, suspected abuse/neglect, exploitation, medical emergency, missing vulnerable participant, or other urgent event.
- PSS is not a first-responder service. Employees shall activate appropriate emergency/crisis resources rather than attempting to independently manage emergencies outside role.
- Complete required incident reports promptly and truthfully. Incident documentation is separate from clinical/service documentation unless policy requires related entries in both locations.
- Do not retaliate against a participant or employee for making a safety, rights, abuse, compliance, or grievance report.

20. Performance Expectations and Progressive Corrective Action

Performance is evaluated using service quality, reliability, documentation accuracy, participant engagement, scope compliance, attendance, training completion, professionalism, productivity, teamwork, safety, and response to supervision.

- ACE may use coaching, documented counseling, retraining, performance-improvement plans, increased supervision, reassignment, suspension from billable duties, written warning, final warning, or separation as appropriate to the circumstances.
- ACE is not required to use every step in sequence when the conduct is serious, involves safety or integrity, or otherwise warrants accelerated action.
- Serious integrity violations may include falsified records, fraudulent billing, privacy breaches, credential misrepresentation, retaliation, workplace violence, theft, diversion, intentional time falsification, or knowingly billing a service not provided.

21. Fraud, Waste, Abuse and False Claims Prevention

- Never submit or cause submission of a claim known to be false, unsupported, duplicated, inflated, or based on a service not rendered.
- Report suspected overpayments, duplicate billing, credential issues, documentation falsification, or billing-system errors immediately.
- Do not alter clinical/service documentation at the request of billing staff merely to make a claim payable. Billing questions must be resolved through the compliance/QP review process.
- ACE prohibits retaliation for good-faith reporting of suspected fraud, waste, abuse, privacy, safety, or regulatory concerns.

22. Conflicts of Interest, Gifts and Outside Employment

- Disclose any financial, family, romantic, supervisory, vendor, referral, or outside-employment relationship that could affect impartial job duties.
- Do not accept kickbacks, referral fees, cash, gift cards, or anything of value in exchange for referrals, participant steering, billing decisions, or vendor selection.
- Outside employment may not interfere with ACE duties, use ACE information/resources, create a conflict, or involve unauthorized solicitation of ACE participants or employees.

23. Social Media, Public Statements and Photography

- Do not post participant information, photographs, recordings, schedules, locations, case details, or stories that could identify a participant without explicit organizational authorization and legally sufficient permission.
- Employees may not speak on behalf of ACE to media, regulators, elected officials, or public audiences unless authorized. This does not limit legally protected employee activity.
- Personal social-media use must not imply ACE endorsement, disclose confidential information, harass coworkers/participants, or interfere with required work.

24. Substance-Free Workplace and Smoking/Vaping

Employees must report to work fit for duty and may not possess, use, distribute, or be impaired by illegal drugs or unauthorized controlled substances while working, driving for ACE, providing services, or using ACE property. Alcohol impairment during work is prohibited. Smoking/vaping is permitted only in authorized areas and never during participant service in violation of site rules.

Lawfully prescribed medication is not prohibited, but an employee must address work restrictions through the appropriate confidential process if medication may impair safe performance of essential duties.

25. Workplace Violence, Threats and Weapons

Threats, intimidation, stalking, physical violence, deliberate property damage, and other conduct creating a reasonable safety concern are prohibited. Employees must promptly report credible threats or unsafe conditions. Weapons are governed by applicable law and ACE site/property rules; no employee may use possession of a weapon to intimidate another person.

26. Complaints, Reporting and Non-Retaliation

Employees are expected to raise compliance, safety, privacy, billing, workplace, participant-rights, or ethical concerns promptly. Reports may be made to the immediate supervisor, Program Manager, Executive Director, QP/clinical leadership, or another designated channel when the normal reporting line presents a conflict.

- Good-faith reporting is protected from retaliation.
- Knowingly false reports or deliberate misuse of the complaint process may be addressed through performance procedures, but an unsubstantiated good-faith report is not misconduct.
- Employees must cooperate honestly with authorized investigations and preserve relevant records.

27. Remote and Hybrid Work

- Remote work is a work arrangement, not an entitlement, and may be modified based on participant needs, performance, privacy, security, or operational requirements.
- Maintain a private workspace appropriate for confidential calls, documentation, telehealth, and meetings.
- Do not permit household members to access ACE devices, files, calls, recordings, or participant information.
- Use approved secure internet access and avoid public Wi-Fi for PHI unless ACE has provided an approved secure method.
- Remote employees remain subject to scheduling, timekeeping, attendance, meeting, supervision, productivity, documentation, and dress/appearance expectations.

28. Leave, Accommodations and Benefit Plans

Eligibility for paid time off, holidays, insurance, retirement, leave, and other benefits is determined by applicable plan documents, written employment terms, employee classification, and law. Benefit plan documents control when they conflict with a handbook summary.

Employees requesting disability, pregnancy, religious, military, family/medical, or other protected leave/accommodation should contact the designated administrative representative as early as practicable. ACE will engage in required processes and maintain confidentiality consistent with law.

29. Separation, Return of Property and Access Termination

- Upon separation or request, return ACE phones, keys, identification, equipment, records, payment cards, access tokens, and other property.
- Employees may not retain participant information, recordings, contact lists, credentials, or copies of ACE records after separation.
- ACE may immediately disable system access when required for security, privacy, safety, or business-continuity reasons.
- Final pay and benefit continuation will be administered according to applicable law and plan terms.

30. Handbook Administration and Changes

The Executive Director and Program Manager are responsible for handbook administration, with QP/clinical leadership responsible for clinical and Medicaid-related workforce requirements within their scope. Material changes should be reviewed through ACE's policy-governance process and communicated to affected staff.

Employees are responsible for complying with the most current approved version. Training, acknowledgment, or policy updates may be distributed electronically. Failure to sign an acknowledgment does not eliminate an employee's obligation to follow communicated policies.

Appendix A. Billable Service Note Checklist

#	Pre-Billing Control
1	Beneficiary identity and Medicaid ID are correct.
2	Date of service is the actual date service occurred.
3	Service type and contact modality are identified.
4	Place of service is documented.
5	Purpose of contact directly relates to a current PCP goal.
6	Intervention describes what the CPSS actually did and supports the time billed.
7	Actual start and end times are recorded.
8	Participant response/effectiveness and progress or barriers are documented.
9	Note is signed/dated by the person who delivered the service with credentials/job title.
10	Eligibility, authorization, credentialing, and telehealth/telephonic limits are satisfied.
11	No duplicate, overlap, impossible-time, cloned, pre-documented, or backdated content exists.
12	If a designated recorded-call workflow applies, recording exists or an approved exception is documented.

BILLING HOLD RULE

If any required element is missing or inconsistent, the encounter remains on billing hold until the issue is resolved through an authorized, truthful correction process. If it cannot be corrected lawfully, the encounter remains nonbillable.

Appendix B. Call Recording Disclosure and Exception Procedure

Approved Disclosure

"This call may be recorded for quality assurance, training, documentation, and compliance purposes. Before we continue, do I have your consent to proceed with the recorded call?"

If Consent Is Declined

1. Stop or do not initiate the recording.
2. Explain that ACE will use an approved alternative workflow when available.
3. Do not pressure the caller to consent.
4. Document the recording exception in the approved system without placing unnecessary PHI in administrative notes.
5. Notify the supervisor if the call is tied to a service workflow that ordinarily requires recording.

Technical Failure

- Do not recreate or fabricate a recording.
- Document the technical exception and report the system issue.
- Complete the required Medicaid service note based on the actual encounter.
- Place the encounter on compliance review if ACE policy requires recording for that workflow.

Appendix C. Telehealth Session Checklist

Stage	Employee Control
Before	Confirm payer/service eligibility; approved platform; privacy; participant identity; participant location; consent/privacy preferences; emergency plan as required.
During	Professional setting; no driving; no unauthorized person present; protect screen/audio; deliver goal-linked intervention within scope; do not record unless separately authorized and consented.
After	Document modality, place of service, goal, intervention, start/end time, participant response/progress, signature/credentials; report incidents; ensure utilization limits are monitored.

Appendix D. Training, Supervision and Audit Calendar

Frequency / Deadline	Required Activity	Owner / Participants
At hire / before access	Confidentiality, cybersecurity, systems access, handbook orientation, role/scope review	Administration / Supervisor / Employee

Frequency / Deadline	Required Activity	Owner / Participants
Within 30 calendar days	PSS policy review; documentation training; opioid-antagonist administration; supervisor training as applicable	PSS staff / PSS Program Supervisor
Within 60 calendar days	Person-Centered Thinking and PCP/crisis-plan instructional training for PSS Program Supervisor	PSS Program Supervisor
Every other week minimum	Individual supportive/administrative supervision	QP/PSS Program Supervisor + PSS program staff
Weekly / as scheduled	Peer operations huddle	Peer Coordinator / Program Manager / CPSS as assigned
Monthly	Medicaid/compliance meeting and documentation audit review	Program Manager, QP, Claims, Intake, Peer leadership, affected staff
Quarterly	Leadership/board compliance review	Executive leadership / Board as scheduled
Annually	Population-related continuing education + ACE compliance refresher	Applicable staff

Appendix E. Regulatory References

NC Medicaid Clinical Coverage Policy 8G, Peer Support Services

Amended September 1, 2026. Governs PSS eligibility, service delivery, documentation, staff requirements, training, telehealth/telephonic limits, billing units, and audit expectations.

https://medicaid.ncdhhs.gov/8g-peer-support-services

North Carolina General Statutes § 15A-287

Addresses interception/recording of wire, oral, or electronic communications and consent requirements under North Carolina law.

https://www.ncleg.gov/enactedlegislation/statutes/html/bysection/chapter_15a/gs_15a-287.html

U.S. Department of Labor - Fact Sheet #22, Hours Worked under the FLSA

Explains compensable hours worked, including required meetings and training, and off-the-clock work.

https://www.dol.gov/agencies/whd/fact-sheets/22-flsa-hours-worked

U.S. Department of Labor - Fact Sheet #53, Health Care Industry and Hours Worked

Provides health-care examples on compensable training and travel time.

https://www.dol.gov/agencies/whd/fact-sheets/53-healthcare-hours-worked

HHS Telehealth Privacy and Security Guidance

Addresses telehealth privacy/security, risk analysis, secure technology, patient consent/preferences, and recording considerations.

https://telehealth.hhs.gov/providers/best-practice-guides/privacy-and-security-telehealth/develop-privacy-and-security

HHS HIPAA Audio-Only Telehealth Guidance

Explains reasonable safeguards and Security Rule considerations for audio-only telehealth technologies.

https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/hipaa-audio-telehealth/index.html

F. Employee Acknowledgment

I acknowledge that I received access to the ACE Transitions Employee Handbook and understand that I am responsible for reading and complying with the policies applicable to my position. I understand that this handbook is not an employment contract and may be revised as permitted by law.

I understand the organization's requirements regarding company cell phones, recorded business calls, telehealth privacy, mandatory training, required meetings and supervision, Medicaid documentation, audit cooperation, professional dress, confidentiality, and billing integrity.

I specifically understand that a service note that is missing, non-goal-oriented, unsupported, inaccurate, or otherwise noncompliant may not be submitted as a Medicaid claim or counted as billable productivity. I also understand that I must accurately record all hours I actually work when I am a nonexempt employee, including required documentation and corrections.

I understand that I may ask questions about any policy before signing this acknowledgment.

Employee Name: _____

Employee Signature: _____ Date: _____

Job Title: _____

Supervisor/Witness: _____ Date: _____